



PW 233: PSW CARE ACROSS THE LIFESPAN

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NURSING CARE PROCESS REVIEW

FOCUS ON HEALTH PROMOTION

- Assisting to maintain or obtain optimal health
- Accomplished through nursing process
 - Assessment, diagnosis, outcome criteria, process criteria, implementation, evaluation
- **Health promotion:** assessment of strengths, not only deficits
 - Overarching goal

HEALTH PERCEPTION— HEALTH MANAGEMENT PATTERN

- An individual's perceived health and well-being
 - Affects lifestyle and ability to function
 - Provides clues to improve or maintain optimal quality of life
- Assessment
 - Current health and safety practices, previous pattern adherence, use of health care system, health care access, health perceptions and beliefs
- Past health management is a predictor
 - Identify and address reason for failures of past interventions (e.g., belief systems, finances, literacy)

FUNCTIONAL HEALTH PATTERNS: ASSESSMENT OF THE INDIVIDUAL

Gordon's (2016) framework

- Concept of functional patterns and pattern interaction
- Depicts individual's lifestyle
- Holism and totality of person's interaction with environment from foundation for health patterns
- Individual is viewed as whole being using interrelated behavioural areas

GORDON'S 11 FUNCTIONAL HEALTH PATTERNS

Health perception–health management pattern

Nutritional-metabolic pattern

Elimination pattern

Activity-exercise pattern

Sleep-rest pattern

Cognitive-perceptual pattern

Self-perception–self-concept pattern

Roles-relationships pattern

Sexuality-reproductive pattern

Coping–stress tolerance pattern

Values-beliefs pattern

FAMILY HEALTH PROMOTION

- A person's health is strongly shaped by their family environment
 - Need to consider the family they live in
- Why they are connected:
 - Shared habits
 - Support system
 - Health beliefs
 - Care and decision-making
 - Environment

THROUGHOUT
THIS LECTURE,
REFLECT ON...

- Which family structure most closely resembles your own?
- What are some of the dynamics in your family?
- What are some of the roles and responsibilities that you have within your family?

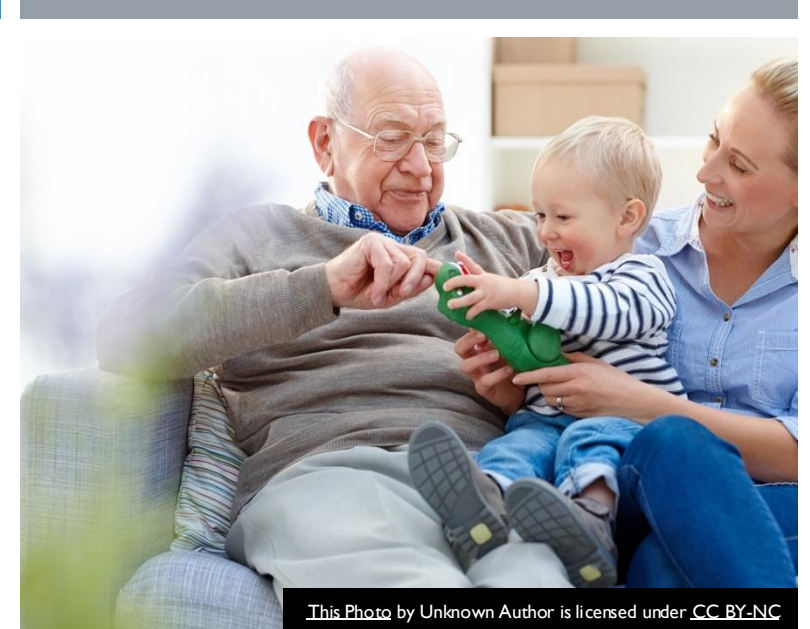
THE FAMILY



- Family: interacting individuals related by blood, marriage, cohabitation, or adoption
- A basic human social system
- The strongest and most influencing group to which a person belongs

THE FAMILY

- Family can take many forms
- Some may consist of people who are related by blood or marriage, or may include people who are unrelated, but have developed a very close relationship with one another
- **Family is whomever the client says they are**



ROLE AS A PSW

- **Ensure** you respect whatever your client's definition of family is
- **Respect** your client's decision as to who they regard as being in their family
- **Refrain** from imposing your own beliefs and values on your client

SCENARIO



- Maria is an 82-year-old client receiving home care. She lives with her neighbor, who she refers to as her “daughter,” even though they are not biologically related. Her biological children live out of province and rarely visit.
- Discussion Questions:
 - Who is considered Maria’s family?
 - How should you, as a PSW, respond to her definition of family?
 - Why is it important not to impose your own beliefs about family?

HEALTHY FAMILY



- Does **not** mean that the members are at an **optimal** level of health and well being
- A healthy family is a family that successfully adapts for challenges, changes, and crisis that can occurring during the life cycle

CHARACTERISTICS OF A HEALTHY FAMILY

- Nurturing Relationships
 - Maintains trust and shares quality of time
 - Open communication
- Establishing Routines
 - Maintains routines that promote health patterns, safety, disease prevention
- Maintaining Expectations
 - Maintains morale and motivation, loyalties, and values



CHARACTERISTICS OF A HEALTHY FAMILY

- Adapting to Challenges
 - Evolves during crises and respects each member
 - Promotes strategies to make decisions about health and illness
 - Shares power appropriately
- Connecting to the Community
 - Members act as interactive caregivers across the life span to socialize children and adolescents, to participate in the community, and to support members as they age



FUNCTIONAL VS DYSFUNCTIONAL FAMILY

Functional Family	Dysfunctional Family
• stable family unit • consistent rules whose members can deal with conflict, stress, and problems in a way that promotes the physical and psychological well-being of its members	• a family unit with inconsistent members or rules • exhibits poor interpersonal relationships among its members, and • deals poorly with conflicts or issues

INFLUENCE OF THE FAMILY



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- Interdependently perform functions via fulfilling expected roles
- Enduring influence on health: individual and family
 - Patterning diet, activity, hygiene, coping
 - Providing resources
 - Fostering resilience
 - Individualization within family cohesiveness
- Influence children's lifestyle choices

FAMILY STRUCTURES

- The composition of families has changed dramatically over the years
- The traditional nuclear family has given way to various types of dynamic contemporary family structures

CONTEMPORARY FAMILY STRUCTURES



- Single parent (separated, divorced, or widowed)
- Unmarried single parent (never married)
- Unmarried cohabitating couple
- Unmarried parents
- Commune family
- Stepparents

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CONTEMPORARY FAMILY STRUCTURES



- Adoptive Family
- Family of Choice
- Married Couple
- Same-Sex Couple
- Married Parents
- Nuclear Family



CONTEMPORARY FAMILY STRUCTURES

- Gay, lesbian, transgender
- Immigrant Family
- Biracial or Multiracial family
- Transracial Family
- Blended-Family
- Joint-Custody Family

CONTEMPORARY FAMILY STRUCTURES

- Conditionally separated families
- Extended Family
- Foster Family
- Grandparent(s)
- Dual-Career



CONTEMPORARY FAMILY STRUCTURES: DUAL CAREER FAMILIES

- Both parents are employed outside of the home
- Some challenges are:
 - Childcare arrangements
 - Finding time for children to participate in activities
 - Finding time to support and encourage academics
 - Finding time to support and encourage interaction with peers
 - Finding time for family activities geared towards the children
 - Lack of energy for home and child-care activities
 - Stresses associated with unexpected illnesses or injuries
 - Expectation for child to be independent and manage self
 - Maintaining health meal options

COUPLE FAMILY



- The first stage of family development typically begins when two adults define themselves as a family
- By moving from their family of origin into this new couple relationship, individuals must now adapt to the role expectations of their partner

COUPLE FAMILY

- Major adjustment
- Adapting to expectations of a partner
- Weave individual characteristics into “couple”



COUPLE FAMILY

- New couples are also faced with numerous decisions that may be made consciously or unconsciously:
 - Who is employed and where?
 - Who manages money, and how will it be managed?
 - Where do they live?
 - How will they socialize with family and friends?
 - Will they have children? When and how many?
 - How will they divide household tasks? (cooking, washing, cleaning, shopping, etc)
 - How will they manage conflict?

CHILDBEARING FAMILY



- Adjust to new roles in function and responsibility
- Explore ways to meet needs
- Influenced by family of origin
- Single-parent family is becoming more common

CHILDBEARING FAMILY



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- First time parents:
 - May feel a lack of emotional support during the first few months
 - Need support
 - May need assistance to grow in their parenting role
- Employment and financial situations must be considered:
 - Childcare
 - Adequacy of income or finances



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CHILDBEARING FAMILY

- Risks:
 - insufficient resources
 - abuse or neglect
 - immature parents
 - beliefs regarding punishment and obedience

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CHILDBEARING FAMILY



- Parental influence on a child begins at birth- single most important factor in the child's:
 - Physical Development
 - Emotional Development
 - Cognitive Development

CHILDBEARING FAMILY



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- Assuming responsibility for their child's development depends on a variety of factors:
- How they were nurtured as children
- Conceptions about self, culture, social class, and religion
- Relationships
- Values and philosophy of life
- Perceptions of and experiences with children and other adults
- Experienced life stresses

FAMILIES WITH TODDLERS AND PRESCHOOLERS



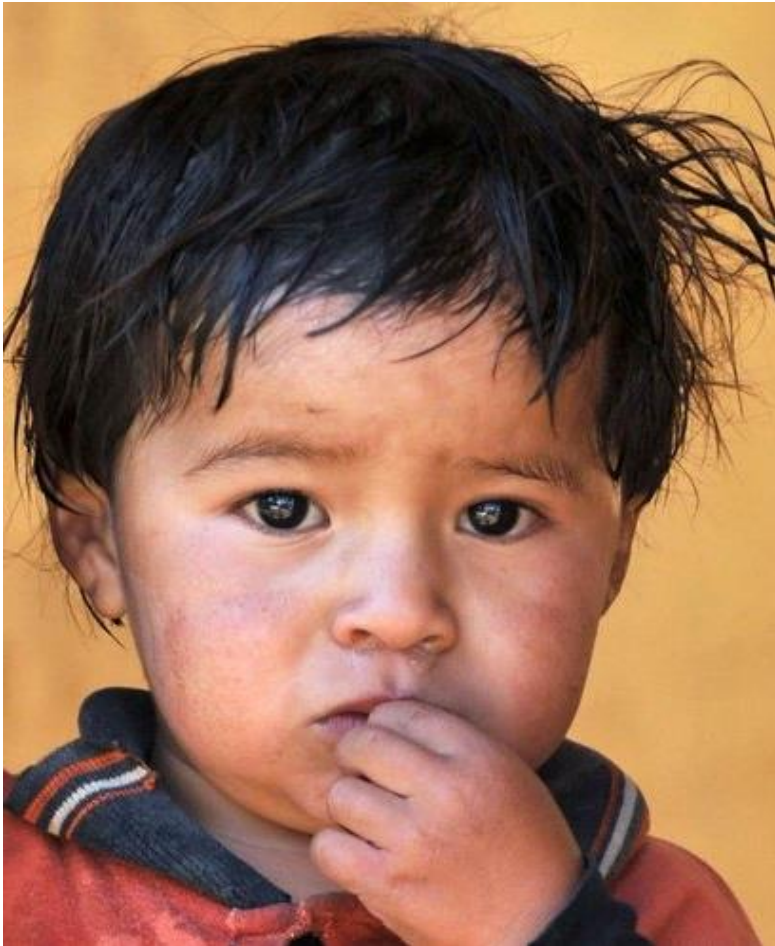
- the needs and interest of those children often influence the home environment
- Balance home safety with child exploration
- Search for ways to **include** young children in stimulation-promotion opportunities
- Children in this age group learn from modelling and use of positive reinforcement

FAMILIES WITH TODDLERS AND PRESCHOOLERS



- Parents to teach and model health behaviours
 - Diet, hygiene, food, sleep, dental care
- Development influenced by religion, ethnicity, education, discipline techniques

FAMILIES WITH TODDLERS AND PRESCHOOLERS



- There is a strong link between a child's home environment and their overall health.
 - Contaminated air – secondhand smoke
 - Contaminated water – lack of, or unclean water
 - Contaminated food – lack of, improper refrigeration
 - Lead paint

FAMILIES WITH TODDLERS AND PRESCHOOLERS



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- Parents risk fatigue from parenting demands
 - Need time as a couple, respite
 - Economic restraints may be a limitation

FAMILIES WITH SCHOOL-AGED CHILDREN



- one of the major issues that parents encounter is the dichotomy
- Dichotomy: pursuing self-interests vs. finding fulfillment in producing next generation

FAMILIES WITH SCHOOL-AGED CHILDREN



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- Goal: reorganization to prepare for expanding child's world
 - New relationships with peers at school
 - Extra-curricular activities
 - New interests
- School achievement is venue for socialization

FAMILIES WITH SCHOOL-AGED CHILDREN



- Interventions: educate regarding risks, teach benefits of healthy behaviours
- Developmental task (for both parents and children) is “letting go”

FAMILIES WITH SCHOOL-AGED CHILDREN

- Encourage children to engage in family discussions regarding their culture and heritage in order to foster their understanding of self within their family network



FAMILIES WITH ADOLESCENT CHILDREN



- Goal: loosen family ties
- Promote identity formation, autonomy, responsibility
- individual members have their own developmental tasks and goals
 - Family unit has tasks they need to accomplish

FAMILIES WITH ADOLESCENT CHILDREN

- Open communication can be difficult
 - Differing developmental tasks of parents and children
 - Adolescents: seek their own identity
 - Parents: try to facilitate the adolescent decision-making process
 - Differing choices about values and lifestyle
 - Desire and needs of adolescents may not be in line with parents
 - Difference in opinion on family values and standards

FAMILIES WITH ADOLESCENT CHILDREN



- High-risk behaviours are very common.
- Peer pressure is very significant.
- Adolescents commonly challenge family values.
- Time of identity crisis and adjustment
 - Include in decision-making
 - Allow adolescents to experience positive and negative consequences.

FAMILIES WITH YOUNG ADULTS



- Goal: launch children
- Parents return to marital dyad.
- Redefine relationships—multigenerational

FAMILIES WITH YOUNG ADULTS



- Aging parents and adult children may present financial and emotional challenges.
- Focus of health-promoting activities:
 - Coping with social and occupational pressures
 - Promoting healthy aging
 - Reassessing life goals

FAMILIES WITH MIDDLE-AGED ADULTS



- May consist of only 2 members of the family (children gone)
- **Focus** on self concept
- Support **own** relationship
- Sense of **freedom** and well being
- **Diminished** parenting pressures
- **Established** network of friends or peers
- Economic **security** and self-esteem may be at its prime

FAMILIES WITH MIDDLE-AGED ADULTS



- Empty-nest syndrome is common.
- Marriage status important: secure and stable vs. faltering marriage
- Challenges: grandparent role, needs of middle-aged children, care of aging parents
- Health tasks: awareness of vulnerability to health problems
- High risks: heart disease, CVA, cancer
- Lifestyle: important to reduce risks

FAMILIES WITH OLDER ADULTS



- Retirement: decreased income, adjustment in living standards
- Challenges: safe and comfortable home, adjust to loss of spouse, preparing for end-of-life

FAMILIES WITH OLDER ADULTS



- Goals for health promotion
 - Maintain function and quality of life, limit disability
- Diseases and dysfunction become increasingly common.
- Sexuality: may still experience sexual desire

CULTURAL SENSITIVITY

- A priority for family assessment
- Sociocultural attunement
 - Ongoing process of experiencing clients' emotions around the intersection of sociocultural contexts (i.e., gender, ethnicity, religion, sexual orientation, age, etc.)
- Culturally humble care

FAMILY THEORIES AND FRAMEWORKS: FAMILY SYSTEMS THEORY

- Patterns of living among individuals in family system
- Families have unique culture, value system, history
- Values provide means for interpreting events
- System boundaries control information flow
- Changes in one part (member) affect whole family (unified whole vs. sum of parts)
- Family assessment is tool to foster health promotion



EFFECT OF FAMILY ON GROWTH AND DEVELOPMENT OF THE CHILD

- Various factors can affect the growth and development of a child:
 - Size of family
 - Spacing of Siblings
 - Divorce
 - Stepchildren
 - Foster children
 - Chronic illness



SIZE OF FAMILY



- Multi-child families:
 - often a unique interpersonal relationship that develops between the siblings
 - allow for early experiences in developing the skills necessary for social interactions
- Single child families:
 - child is exposed to a stronger one-to-one type relationship with a parent
 - developing a larger vocabulary and enhanced conversational skills

SPACING OF SIBLINGS



- arrival of a new baby can result in a variety of responses from the sibling(s), depending on the age of the older child:
 - 1-year-old- may be described as whinier and more clingy.
 - 2-year-old- may experience regression in toileting or self-feeding behaviours
 - 4-year-old- may experience an increase in, or new development of, tantrums.
 - 5 years of age or older- typically feels proud and protective.

SPACING OF SIBLINGS



- Sibling rivalry
- Increased level of stress and chaos in the family
- Can positively contribute to a child's development
 - learn conflict resolution and interpersonal skills

DIVORCE



- The psychological health and overall development of a child may be affected if his or her parents divorce
- Children of divorce often have a higher incidence of both learning and behavioural difficulties in their life

DIVORCE



- Move to single-family home, or a newly blended family
- Psychological adjustment of the child
 - Impacted by presence of hostility between the parents
- Children should be encouraged to express their feelings

Age Group	Signs and Symptoms	Interventions
Preschool	Regression Returns to thumb sucking Intensified fears Sleep disturbances Fear of abandonment	Maintain household routines Reassure love of child Spend added time with child Establish and maintain bedtime routines
School-Aged	Shows open grieving Feels rejected Fears being replaced by absent parent Difficulty in concentrating, resulting in poor grades Fears expressing emotions	Allow child to love both parents Help child move from parent to parent Diminish worry about present and future Attend school events Do not use child as confidant or as spouse substitution

DIVORCE

Age Group	Signs and Symptoms	Interventions
Adolescent	Worry about fate of own future and marriage Must rethink values and morality of the world May become depressed or suicidal May be expected to assume greater family responsibilities	Avoid delegating too much home responsibility Encourage discussion with neutral party Encourage pursuit of own interests Help in use of support systems
Adult	Increased dependence on eldest child Open expression of fears, anxieties, and anger Inability to focus on needs of children	Counsel to join a single-parent support group Encourage professional counseling Refer to social or financial-aid resources
Geriatric	Depression, loneliness, helplessness, bitterness Increased dependence on children and grandchildren	Encourage joining groups Increase activities that are of personal interest

DIVORCE

STEPCHILDREN



- one or both of the spouses have children from a previous relationship that they are bringing into their new family unit
- children may undergo significant adjustments:
 - New home
 - New school
 - New rules
 - New roommates (sleep siblings)
 - Less attention from biological parent

FOSTER CHILDREN



- Placed in strange environment
- Children experience uncertainty about:
 - Can they trust these strangers?
 - Where do they fit in?
 - When can they see their family again?

CHRONIC ILLNESS



- Strained relationships
- Additional family responsibilities
 - Can cause resentment or anger
- Overprotective parents over surviving children
 - Deprive children of normal interactions and independence

FAMILY INTERVENTIONS

- Health care team and family work together
 - New info used to change or adapt plan as needed
- Interventions assist with functions that members cannot independently perform
- Assist families to improve capacity to act independently
- Focus of plans is to foster health promotion

EVALUATION WITH THE FAMILY

- Purpose: to determine family response to planned interventions
- Were the interventions successful?
- Five measures of family functioning can guide determining effectiveness of plan:
 1. Changes in interaction patterns
 2. Effective communication
 3. Ability to express emotions
 4. Responsiveness to needs of individual members
 5. Problem-solving ability

NEXT CLASS

Required Readings:

- Edelman & Kudzma (Ch. 11)